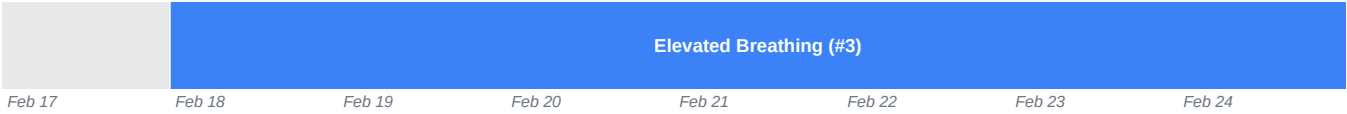


CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: Sallen Period: February 17 to February 24, 2024 Report Date: February 25, 2024 Coverage: 132/192h (58.3%)
Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 8 days from Feb 17 to Feb 24 ■ HR ■ Breathing



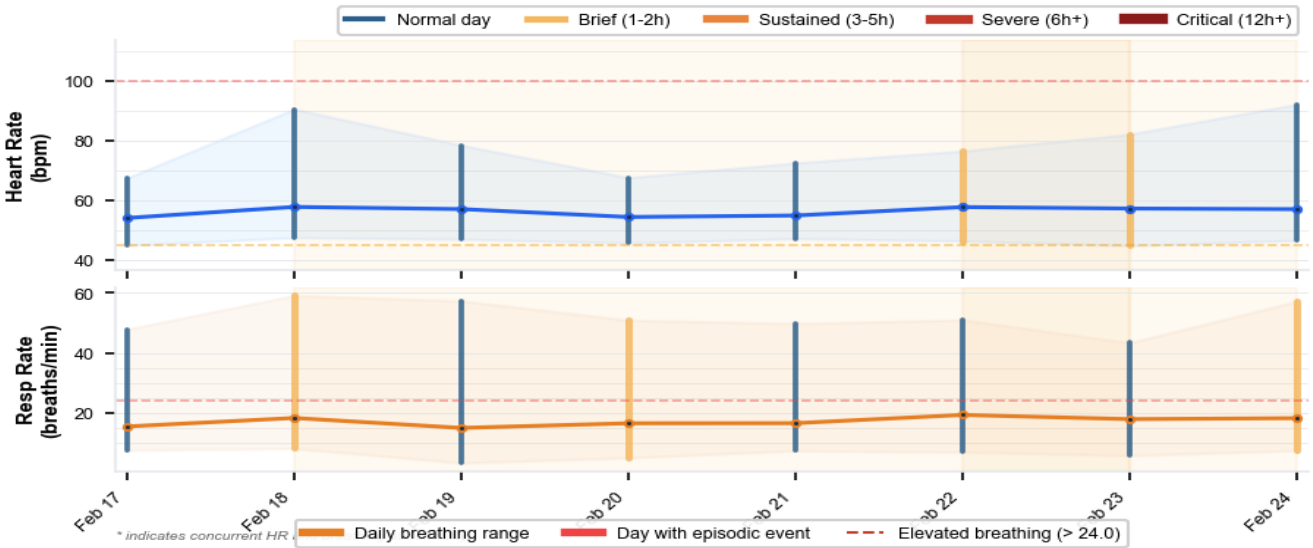
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
10	10h	Very High Breathing	1	69%

Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Very High Breathing	42 brpm	1h	1h	1	18 brpm	Feb 23
2	High Breathing	43 brpm	2h	1h	1	19 brpm	Feb 22 to Feb 23
3	Elevated Breathing	57 brpm	7h	1h	1	17 brpm	Feb 18 to Feb 24

RR values outside physiologic range (6–60 breaths/min).

- Clinical Pattern Observations:
- 1. **Daytime pattern:** Episodes concentrated during waking hours.
 - 2. **Worsening trajectory:** 1 → 10 events vs prior period.



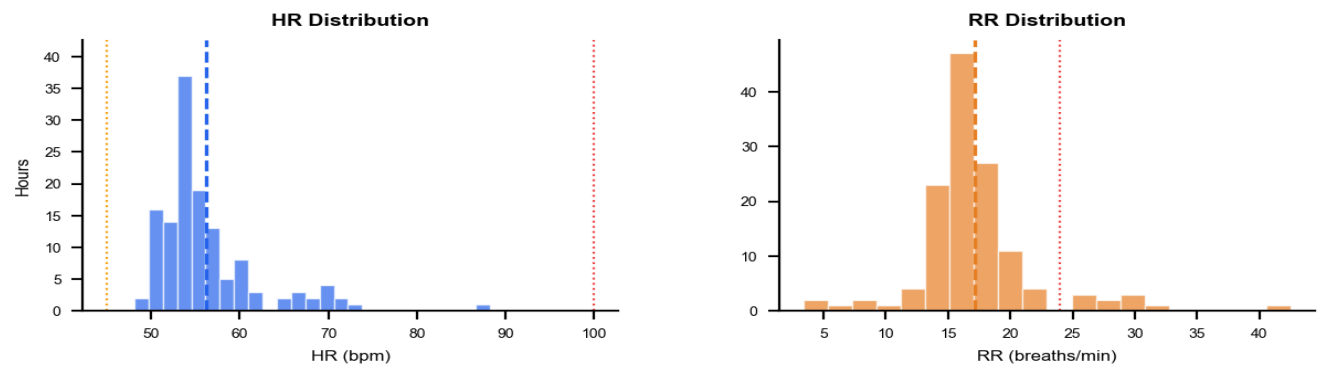
Red bars indicate days with detected episodic events.
■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

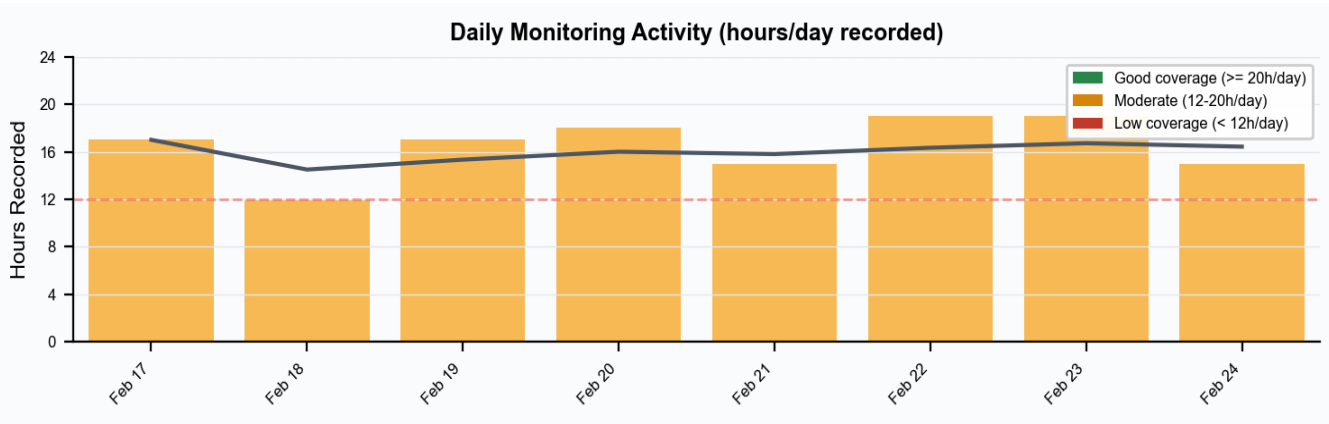
Very Low HR < 40 bpm Low HR < 45 bpm Elevated HR > 95 bpm High HR > 100 bpm
Very High HR > 110 bpm Elevated Breathing > 24 brpm High Breathing > 30 brpm Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.